

Composition:

Each Bezo Film Coated Tablet contains: Bisoprolol Fumarate 2.5, 5, or 10 mg.

Mechanism of Action:

Bisoprolol is a β 1-selective (cardioselective) adrenoceptor-blocking agent. At higher doses (≥ 20 mg) Bisoprolol also inhibits β 2-adrenoreceptors located in bronchial and vascular musculature. To retain relative selectivity, it is important to use the lowest effective dose possible.

Pharmacokinetics:

Bisoprolol is well absorbed after oral administration. Its absorption is not affected by food intake. The absolute bioavailability after a 10 mg dose of Bisoprolol is approximately 80%. The rate of metabolism of Bisoprolol after first hepatic passage is approximately 20%. The binding to serum proteins is approximately 30%. Plasma concentrations reach peak within 2-4 hours of ingestion of a dose of 2.5-20 mg. The elimination half-life is approximately 9-12 hours. Bisoprolol is eliminated equally via the renal and non-renal pathways, with approximately 50% of the administered dose being excreted unchanged in the urine and the remainder in the form of inactive metabolites. Less than 2% of the administered dose is excreted in the feces.

Indications:

Bezo tablets are indicated for the treatment of:

- Hypertension and stable chronic angina.
- Stable chronic heart failure accompanied by decreased left ventricular systolic function, in combination with angiotensin-converting enzyme (ACE) inhibitors, diuretics, and optional cardiac glycosides.

Contraindications:

Bezo is contraindicated patients with:

- Hypersensitivity to the active substance or to any of the excipients.
- Acute heart failure or during episodes of heart failure decompensation requiring IV inotropic therapy.
- Cardiogenic shock.
- Second- or third-degree AV block (without a pacemaker).
- Sick sinus syndrome, sinoatrial block, symptomatic bradycardia and hypotension.
- Severe bronchial asthma or severe chronic obstructive pulmonary disease.
- Severe forms of peripheral arterial occlusive disease or severe forms of Raynaud's syndrome.
- Untreated phaeochromocytoma, and metabolic acidosis.

Warnings & Precautions:

- The treatment of stable chronic heart failure with Bezo has to be initiated with a special titration phase.
- In the chronic heart failure, the initiation and cessation of treatment with Bezo necessitates regular monitoring.
- The symptoms of thyrotoxicosis may be masked under treatment with Bezo.
- In patients with phaeochromocytoma bisoprolol must not be administered until after alpha-receptor blockade.
- Bezo should be used with caution in cases of: bronchospasm, diabetes mellitus with large fluctuations in blood glucose values, strict fasting, on-

going desensitisation therapy (behavioral therapy), first degree AV block, peripheral arterial occlusive disease, general anaesthesia, psoriasis, hypertension or angina pectoris and accompanying heart failure, and Prinzmetal's angina.

Pregnancy & Lactation:

Pregnancy: Pregnancy category C. Bezo should be used during pregnancy only if the potential benefit justifies the potential risk to the fetus.

Lactation: Caution should be exercised when Bezo is administered to nursing women.

Drug Interactions:

- Bezo plasma concentrations are expected to increase during concurrent administration of CYP3A4 inhibitors and decrease during concurrent administration of CYP3A4 inducers.
- Bezo may increase the plasma concentrations of other drugs metabolised by CYP3A4 and possibly those metabolised by CYP2D6.
- Bezo should not be combined with other beta-blocking agents.
- Patients receiving catecholamine-depleting drugs should be closely monitored.
- Mefloquine: Combination with Bezo increases the risk of bradycardia.
- Monoamine oxidase inhibitors (except MAO-B inhibitors): Enhanced hypotensive effect of the beta-blocking agents but also risk for hypertensive crisis.
- Concomitant use with parasympathomimetic drugs may increase atrio-ventricular conduction time and the risk of bradycardia.
- Concomitant use with dihydropyridine (e.g., felodipine and amlodipine) may increase the risk of hypotension, and an increase in the risk of a further deterioration of the ventricular pump function in patients with heart failure cannot be excluded.
- Concomitant use with verapamil and diltiazem negative effect on contractility and atrio-ventricular conduction.
- Class-III antiarrhythmic drugs (e.g. amiodarone) effect on atrio-ventricular conduction time may be potentiated.

Side Effects:

Common side effects: headache, fatigue, exhaustion, urinary tract infection, rhinitis or sinusitis, diarrhea or constipation, dizziness, joint pain, cough, insomnia (trouble sleeping), sleep disturbances, nightmares, nausea (feeling like vomiting), sore throat, coldness or numbness in the hands or feet.

Driving & Using Machines:

Bezo may cause dizziness or fatigue and, therefore, may adversely affect the patient's ability to drive or use machinery.

Dosage & Administration:

Bezo tablets should be taken in the morning and can be taken with food.

- Treatment of hypertension and chronic stable angina pectoris:

Adults: The dosage should be individually adjusted.

It is recommended to start with 5 mg per day. The usual dose is 10 mg once daily with a maximum recommended dose of 20 mg per day.

*Renal impairment: In patients with severe renal impairment (creatinine clearance less than 20 ml/min) the dose should not exceed 10 mg once daily. This dosage may be given in two divided doses.

In elderly patients, it is recommended to start with the lowest possible dose.

- Treatment of stable chronic heart failure:

Adults: Standard treatment of CHF consists of an ACE inhibitor, a beta-blocking agent, diuretics, and when appropriate cardiac glycosides.

Patients should be stable (without acute failure) when Bezo treatment is initiated.

Transient worsening of heart failure, hypotension, or bradycardia may occur during the titration period and thereafter.

*Titration phase: The treatment of stable chronic heart failure with Bezo requires a titration phase.

The treatment with Bezo is to be started with a gradual up titration according to the following steps:

1.25 mg once daily for 1 week, if well tolerated increase to 2.5 mg once daily for a further week, if well tolerated increase to 3.75 mg once daily for a further week, if well tolerated increase to 5 mg once daily for the 4 following weeks, if well tolerated increase to 7.5 mg once daily for the 4 following weeks, if well tolerated increase to 10 mg once daily for the maintenance therapy.

The maximum recommended dose is 10 mg once daily. If the maximum recommended dose is not well tolerated, gradual dose reduction may be considered.

Overdose:**Symptoms:**

The most common signs expected with overdose of a beta-blocker are bradycardia, hypotension, bronchospasm, acute cardiac insufficiency and hypoglycaemia. There is a wide inter-individual variation in sensitivity to one single high dose of Bezo, and patients with heart failure are likely to have increased sensitivity.

Treatment:

In general, if overdose occurs, discontinuation of Bezo treatment and supportive and symptomatic treatment is recommended.

The following general measures should be considered when clinically warranted.

- Bradycardia: Administer intravenous atropine. If the response is inadequate, isoprenaline or another agent with positive chronotropic properties may be given cautiously. Under some circumstances, transvenous pacemaker insertion may be necessary.
 - Hypotension: Intravenous fluids and vasopressors should be administered. Intravenous glucagon may be useful.
 - AV block (second or third degree): Patients should be carefully monitored and treated with isoprenaline infusion or temporary pacing.
 - Acute worsening of heart failure: Administer I.V diuretics, inotropic agents, vasodilating agents.
 - Bronchospasm: Administer bronchodilator therapy such as isoprenaline, beta 2-sympathomimetic drugs and/or aminophylline.
 - Hypoglycaemia: Administer I.V glucose.
- Limited data suggest that Bezo is hardly dialysable.

Storage:

Keep out of reach of children. Store below 25° C.

Packaging:

Each Bezo (2.5, 5, 10) carton box contains 20 Film Coated Tablets in two blister strips.

بيزوبرولول هو حاصر انتقائي لمستقبلات β_1 (انتقائي للقلب). عند تناوله بجرعات أعلى (≤ 20 ملغ)، يثبط بيزوبرولول أيضاً مستقبلات β_2 الأدرينالية الموجودة في القصبات الهوائية والأوعية الدموية. للحفاظ على الانتقائية النسبية، من المهم استخدام أقل جرعة فعالة ممكنة.

- يجب استخدام بيزو بحذر في حالات: التشنج القصبي، داء السكرى المترافق بتقلبات كبيرة في قوتها، غلظت الدم، الحمل، المرض المزمن، المعالجة المستمرة لانزلة الشعبتين (علاج

البلوغ: يتكوّن العلاج القياسي لفشل القلب الاحتقاني من مثبّطات الإنزيم المحوّل للأنجيوتنسين، حاصرات بيتا، مدرّات البول، وعند الحاجة الغليكوزيدات القلبية. يجب

مستحسن يؤثر على صحتك واستعماله خلافا للتعليمات عرضك الخطر
وصفة الطبيب وبطريقة الاستعمال المستحسنين
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